Copilot Efficiency Plan

Facility: Regional Outpatient Clinic
Prepared By: Health Administration Coordinator
Date: October 27, 2026
Target Audience: Dr. Serena Walsh (Clinic Director) and Team Leads
Planning Horizon: Next quarter (Weeks 1–12), measured against the 4-week baseline audit

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Executive Summary

Two of the four evaluated tasks are strong Copilot candidates, ambient encounter-note drafting is conditional on mandatory clinician review, and prior-authorization decisions remain human-led. Next quarter prioritizes Dragon Copilot encounter notes and Microsoft 365 Copilot portal replies, which drive 43.0 of the 76.5 weekly hours lost (56%), aiming to recover about 27 staff hours per week.

Section 1—Automation Suitability Summary

Executive Summary Statement

Of the four tasks evaluated, two are strong candidates (pre-visit messaging and portal inbox triage), one is conditional (ambient encounter-note drafting), and one is not suitable (prior-authorization decisions).

Evaluation Table

Task Name and Ref #	Repeatability Factor	Risk Factor	Review Requirement Factor	Assigned Suitability Rating	Evaluation Rationale (Grounded in Scenario)
Task 1: Pre-Visit Reminder and Preparation Messaging	High	Low	Minimal	**Strong Candidate**	About 120 near-identical messages per week are built from baseline templates. Errors cause mild inconvenience or, at worst, a rescheduled lab draw, and review is a quick check of appointment time and prep flag before sending.

Task Name and Ref #	Repeatability Factor	Risk Factor	Review Requirement Factor	Assigned Suitability Rating	Evaluation Rationale (Grounded in Scenario)
Task 2: Draft Encounter Note Summarization (Dragon Copilot)	High	Moderate to High	High (mandatory)	**Conditional Candidate**	About 150 encounters per week follow a fixed SOAP structure, and manual notes take 8–12 minutes each. However, an omitted symptom, misread dosage or misattributed history affects care quality and legal compliance, so adoption depends on full clinician review before EMR sign-off.
Task 3: Initial Prior-Authorization Submission Decisions	Medium	High	High	**Not Suitable**	Volume is moderate (about 25 requests per week), and each case needs clinical judgment and payer-policy interpretation. Errors cause denials, care delays and financial liability, and final authorization decisions cannot be delegated to AI; Copilot may only summarize records for coordinator review.
Task 4: Daily Patient Portal Inbox Triage and Tagging	High	Low to Moderate	Low to Moderate	**Strong Candidate**	About 200 messages per week are sorted into a fixed tag set (Billing, Urgent Clinical, Refill, Scheduling), and keyword-driven sorting with staff oversight is reliable. The main risk, an urgent symptom tagged as routine, is caught by a rapid tag scan before auto-routing.

Section 2—Time-Loss Analysis and Improvement Opportunity Inventory

Executive Summary Statement

The two prioritized opportunities are post-visit encounter-note generation with Dragon Copilot (Documentation, 24.5 hrs/week) and routine patient-portal response drafting with Microsoft 365 Copilot in Outlook (Communication, 18.5 hrs/week).

Time-Loss Inventory Table

Administrative Work Category	Identified Time-Loss Pattern/Inefficiency	Weekly Hours Lost (Data Value)	Copilot Addressability Rating	Recommended Copilot Tool and Feature
Documentation	**Priority 1:** Manual synthesis of post-visit encounter notes and EMR re-entry of patient-reported history	**24.5 hrs/wk**	**Direct** Generates the SOAP draft from ambient audio	Dragon Copilot: Ambient Clinical Capture and Draft Generation
	Compiling fragmented specialist history from PDF attachments for consultation prep	8.0 hrs/wk	**Partial** Summarizes; staff assemble and verify	M365 Copilot in Word/Teams: Document Summarization
Communication	**Priority 2:** Drafting individualized replies to routine portal inquiries (refill status, prep questions)	**18.5 hrs/wk**	**Direct** Drafts the reply from the incoming message	M365 Copilot in Outlook: Draft with Copilot and Tone Adjustment
	Phone follow-up for missed-appointment rescheduling and confirmation calls	6.5 hrs/wk	**Indirect** Drafts outreach text; calls stay manual	M365 Copilot in Outlook: Batch Scheduling Follow-up
Clerical Work	Daily inbox triage, message sorting and manual assignment to specialty folders	12.0 hrs/wk	**Partial** Suggests tags; staff verify routing	M365 Copilot in Outlook/Teams: Inbox Summarization and Auto-Tagging
	Re-keying referral demographic and insurance data from fax attachments into EMR scheduling fields	7.0 hrs/wk	**Partial** Pre-fills fields; staff check against the fax	M365 Copilot: Form Data Extraction
Total	All six time-loss patterns	**76.5 hrs/wk**	—	**Priorities 1 + 2: 43.0 hrs/wk (56%)**

Prioritization Rationale for Top Two Opportunities

These two were selected because they are the only patterns combining the highest time-loss magnitude with a Direct addressability rating. Encounter-note synthesis is the single largest loss at 24.5 hours per week across six staff, and Dragon Copilot generates the SOAP draft directly from ambient audio, removing the redundant typing that leaves charts unfinalized for 24–48 hours. Its Section 1 Conditional rating does not exclude it; it defines the mandatory clinician review in Specification 1. Portal-response drafting is the second-largest loss at 18.5 hours per week across five coordinators, and Copilot in Outlook drafts replies directly from the incoming message, addressing repetitive typing and inconsistent tone. Together they account for 43.0 of the 76.5 weekly hours lost (56%). Inbox triage ranked third: although a Section 1 strong candidate, its 12.0-hour loss is smaller and its addressability only Partial,

since staff must still verify every tag and route; it is the recommended next-wave opportunity. The remaining patterns are smaller (6.5–8.0 hours) and rated Partial or Indirect.

Section 3—Workflow Refinement Specifications

Specification 1: Post-Visit Encounter Note Generation (Documentation)

- **Target Tool:** Microsoft Dragon Copilot (ambient clinical capture and SOAP draft generation into the EMR chart).
- **Standardized Copilot Prompt** (saved as the clinic's standard note instruction)**:**

> **Task Scope:** Draft a SOAP note for this single outpatient encounter from the ambient recording only; do not add diagnoses, orders or medications that were not stated.
>
> **Context:** Primary care visit at Regional Outpatient Clinic; use the ambient audio, the visit reason, and the patient's EMR medication and problem lists.
>
> **Output Format:** Clinic SOAP template: Subjective (HPI, key patient quotes), Objective (vitals, exam), Assessment (numbered problems), Plan (per problem: medication, dose, route, frequency, follow-up); one page maximum.
>
> **Review Instruction:** Tag [VERIFY] on any unclear dosage, number, laterality or statement and on history attributed to anyone but the patient; label the note "DRAFT – clinician review required."

- **Review Step Checklist (Risk-Proportionate):** Section 1 rated this task Conditional (moderate–high risk), so all four checkpoints are mandatory and completed by the treating clinician.
  - ☐ **Checkpoint 1:** Every symptom raised appears in Subjective, and each Assessment problem has a matching Plan entry.
  - ☐ **Checkpoint 2:** Medication names, doses, routes and frequencies match the visit and the EMR medication list; all [VERIFY] tags are resolved; history is attributed to the correct person.
  - ☐ **Checkpoint 3:** SOAP structure and terminology are correct, no third-party or off-topic conversation is retained, and ambient-recording consent is documented.
  - ☐ **Checkpoint 4:** Only the treating clinician signs (sign-off gate); medical assistants may reconcile formatting but cannot finalize.
- **Handoff Timing Trigger:** Capture starts when the clinician enters the exam room; the draft is handed to the clinician immediately upon exam closure in the EMR, and sign-off is completed within 4 hours of exam closure.
- **Measurable Efficiency Target:** Reduce charting time from 10–12 minutes to under 4 minutes per note (about 15 hrs/week recovered); 100% chart sign-off within 4 hours of exam closure (baseline 24–48 hours); fewer than 5% of signed notes needing a

correction addendum. Measured weekly via EMR timestamps and a 10-chart audit; met by Week 12.

Specification 2: Patient Portal Message Drafting and Communication (Communication)

- **Target Tool:** Microsoft 365 Copilot in Outlook (Draft with Copilot and tone adjustment).
- **Standardized Copilot Prompt** (saved as a shared front desk team prompt)**:**

> **Task Scope:** Draft a reply to this one routine portal message about refill status, appointment details or visit preparation; give no clinical advice or result interpretation.
>
> **Context:** Use the patient's message and these EMR details: [appointment date/time, refill status, prep instructions]. Sender: Regional Outpatient Clinic front desk.
>
> **Output Format:** Under 120 words: greet by first name, answer in the first two sentences, list next steps as bullets, and close with the clinic phone number and signature; warm, plain language (6th-grade level).
>
> **Review Instruction:** Use only the facts provided and insert [CONFIRM] where a detail is missing. If the message mentions new symptoms, pain or side effects, output "ESCALATE – Urgent Clinical" instead of a reply.

- **Review Step Checklist (Risk-Proportionate):** Routine replies resemble the low-risk communication rated Strong in Section 1 (Tasks 1 and 4), so one 30-second coordinator review suffices; only escalations reach clinical staff.
  - ☐ **Checkpoint 1:** Dates, times, refill status and prep instructions match the EMR, and every [CONFIRM] placeholder is resolved.
  - ☐ **Checkpoint 2:** No clinical advice is given; any symptom, side effect or urgent wording is routed to the Urgent Clinical queue instead of being answered.
  - ☐ **Checkpoint 3:** Sent only via the secure portal to the verified patient, with minimum necessary PHI and an empathetic, on-template tone.
- **Handoff Timing Trigger:** Copilot drafts the reply upon opening a new unassigned portal ticket in Outlook, after EMR details are pulled; the coordinator reviews and sends it before opening the next ticket and forwards ESCALATE outputs immediately.
- **Measurable Efficiency Target:** Reduce handling time from 7 minutes (6–8 minute baseline) to under 2.5 minutes per message (about 12 hrs/week recovered), with fewer than 5% of sent replies requiring a correction follow-up. Measured weekly via Outlook and portal timestamps and a 20-message audit; met by Week 12.

Plan Verification Checklist (Self-Audit Before Submission)

- ✓ Section 1 addresses all 4 tasks across all 3 factors. — **Met:** all four tasks are rated on repeatability, risk and review requirement.
- ✓ Section 1 rationale uses specific scenario evidence. — **Met:** each rationale cites the scenario's volumes, risk consequences and review requirements.
- ✓ Section 2 includes time loss figures directly from Document 2 data. — **Met:** all six weekly figures (76.5 hrs/wk total) are taken directly from the audit data.
- ✓ Section 2 covers Documentation, Communication, and Clerical categories. — **Met:** two patterns per category, six in total.
- ✓ Section 3 prompts follow full 4-part anatomy (Scope, Context, Format, Review Instruction). — **Met:** both prompts include Task Scope, Context, Output Format and Review Instruction.
- ✓ Section 3 handoff timing uses operational workflow triggers. — **Met:** triggers are exam closure in the EMR and opening a new unassigned portal ticket in Outlook.
- ✓ Section 3 efficiency targets are specific and measurable. — **Met:** time per task, sign-off window and correction-rate targets, tracked weekly against a Week 12 deadline.
- ✓ Total word count across narrative sections is within **700–900 words**. — **Met:** narrative sections total 863 words (executive summaries, prioritization rationale and Section 3 specifications, excluding headings, field labels and tables).